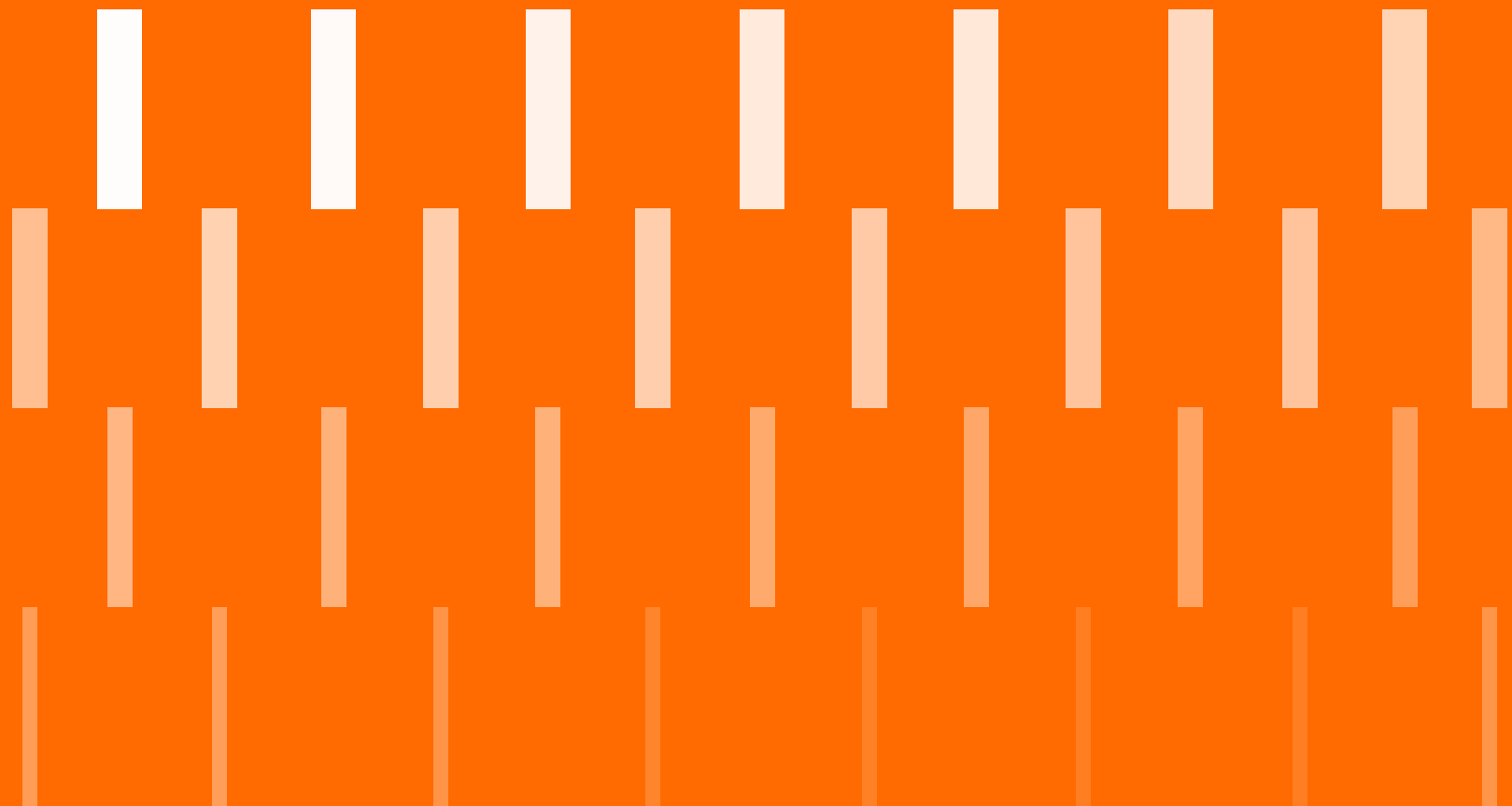




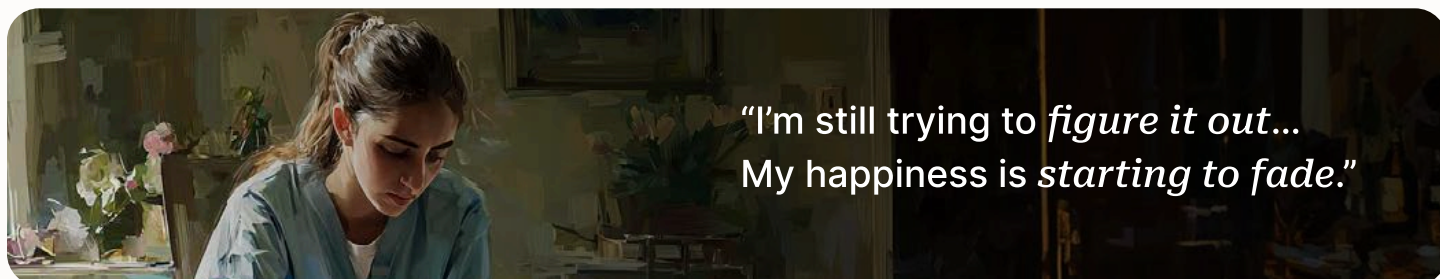
Caregiver Data *Benchmark Report*

What caregivers actually think, feel, and need—in their own words.



The Big Picture

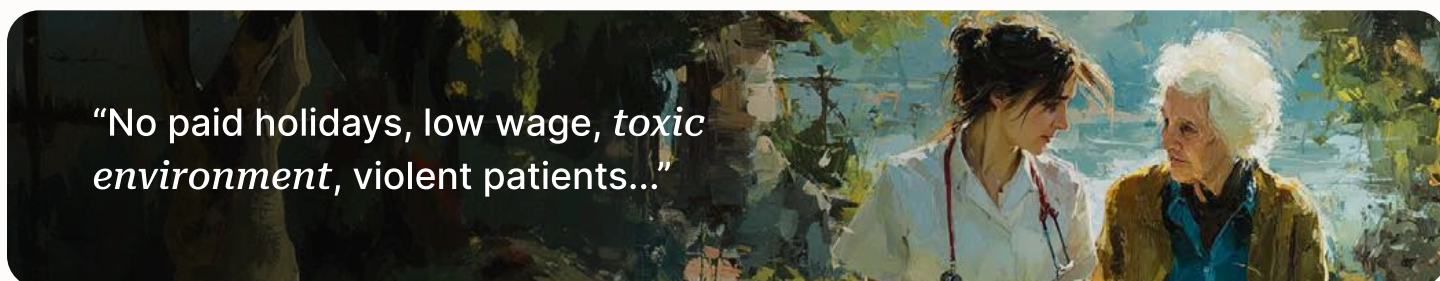
Right now, somewhere in your agency, a caregiver is clocking in, caring for clients, and quietly wondering how much longer she can keep doing this:



She's not alone. Our data shows that one in four caregivers aren't confident they'll still be with their agency in 12 months, and this isn't exit-interview data from people already out the door. It's what's running through people's minds while they're still on the job.

What this report reveals is that caregiver turnover doesn't usually happen because of one bad day. It happens because problems accumulate and nobody intervenes. A caregiver who's financially stressed but feels supported and trusts leadership can manage.

Add a second issue and confidence starts to erode. Add a third and the math changes entirely:



By the time someone actually walks away, the decision isn't fresh. Among caregivers who have left an agency, 37% had already made up their mind weeks earlier, and another 11% said the departure was sudden. By the time the office finds out, the outcome was set long ago.

The good news is that the signals show up early, and the strongest predictors of whether someone stays or goes are things agencies can actually control: how supported caregivers feel day to day, how schedule changes are communicated, whether leadership follows through on commitments, and whether the agency creates an environment where people feel safe enough to speak up before things reach a breaking point.

Whether a Caregiver Feels *Supported* Predicts Everything Else

If there is one finding that runs through this entire report, it's this: caregivers who feel supported by their agency stay, and caregivers who don't will leave. That dynamic is most intense in the first year, when the relationship between caregiver and agency is still being formed.

Caregivers in their first three months at an agency show a 47% at-risk rate for leaving, nearly half. That drops after the initial period but never fully stabilizes: it holds around 25% through the rest of the first year. The early weeks, when a new caregiver is figuring out whether the reality matches what they were told, are by far the most dangerous moment in the relationship.

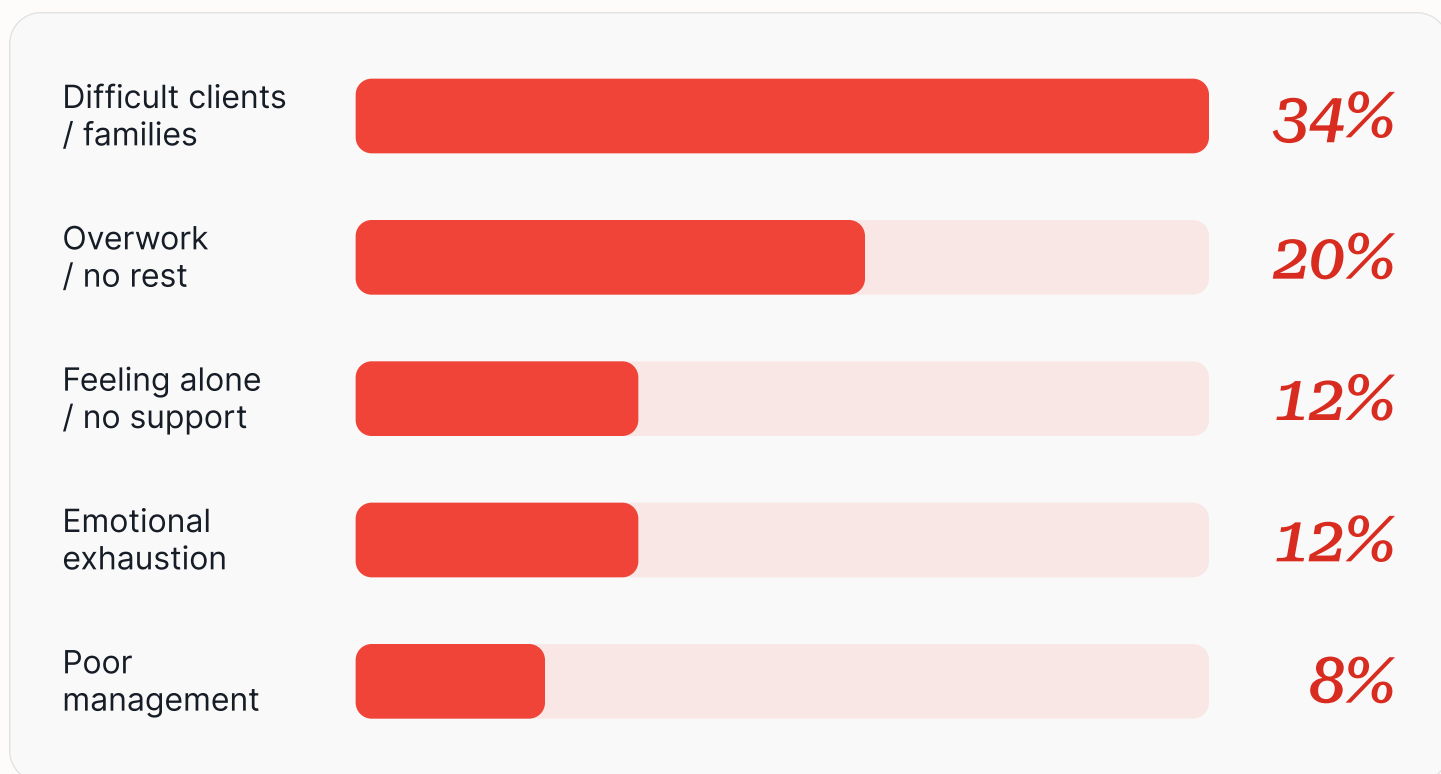


Some caregivers described that early period as being set up to fail:

“

“Months of CNA training thrown into only 8 hours is *not enough*.
I felt as though I was thrown to the wolves.”

What caregivers say *drives burnout*



Difficult clients and family dynamics lead the list by a wide margin, and caregivers describe a reality that most office staff will never fully see. They're walking into unfamiliar homes, working with people who may not want them there, managing behavior that ranges from rude to physically violent, and absorbing emotional weight that accumulates over months and years.



"I work with a *verbally combative* dementia client. Sometimes it takes a toll on me when he has a *really bad day*, or sometimes not *sleeping well* and having to work."

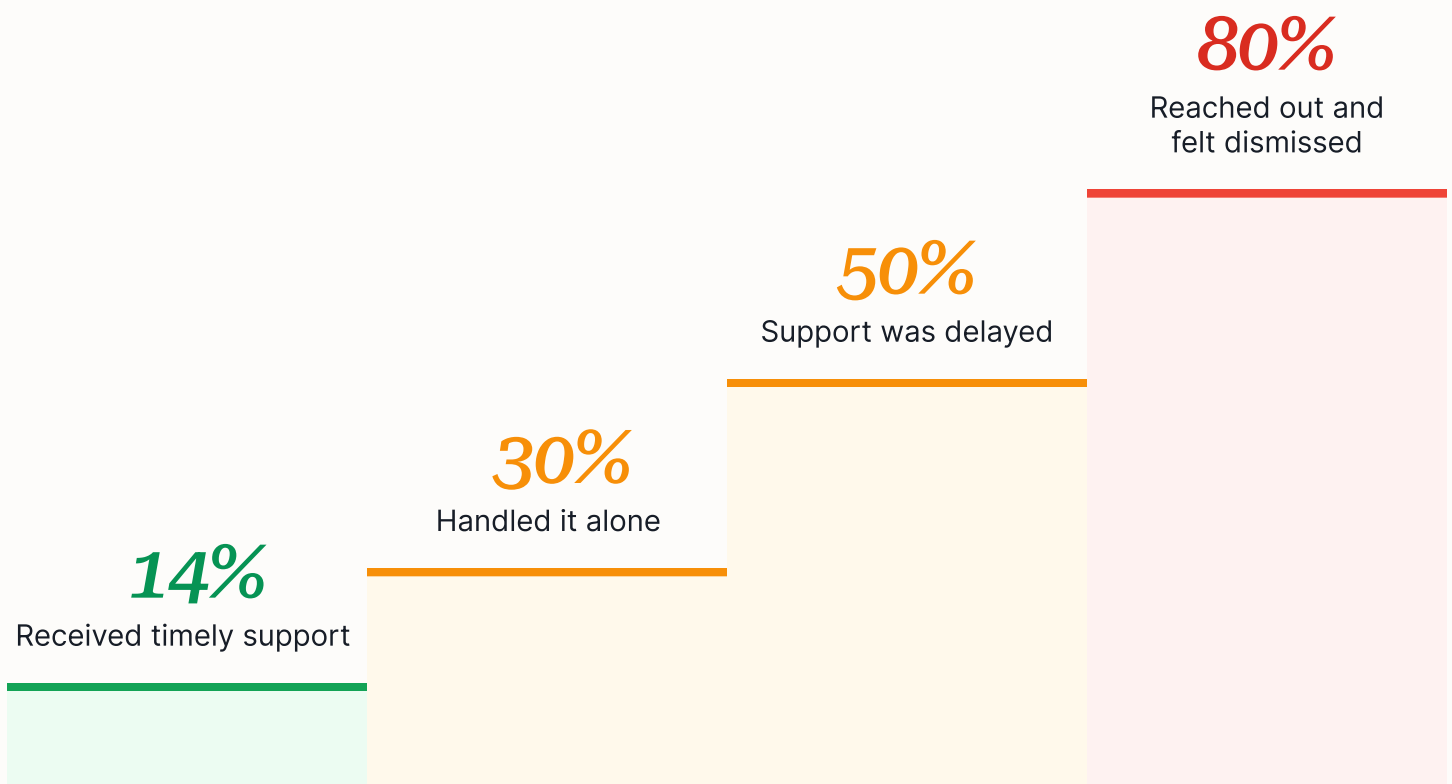
And the difficulty isn't limited to clients. Family dynamics add another layer — "when family members don't acknowledge what you're doing and keep demanding and showing no respect."

Most caregivers accept that this is part of the work. What they don't accept is feeling like the agency disappears when things get hard. The third item on the burnout list, feeling alone with no support, is the one agencies can actually fix, and when they don't, caregivers notice:

“People acting like they *listen and care* and then they *disappear* and *nothing gets done* about the situation.”

The data confirms exactly that. The more supported caregivers feel when things go wrong, the more likely they are to stay, and the relationship follows a clear gradient with no ambiguity:

At-risk rate by *support experience*



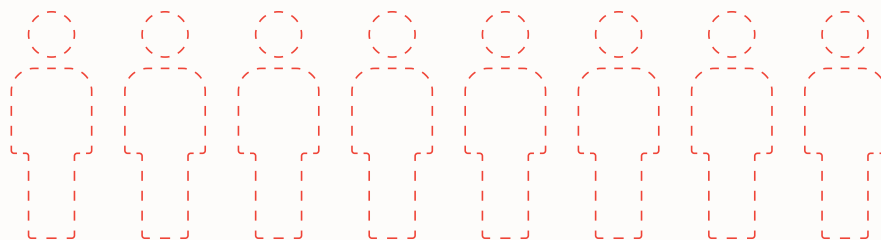
Caregivers who reached out for help and felt dismissed have an 80% at-risk rate, and not a single one is very confident about staying. These are caregivers who did the right thing, asked for help, and learned that asking just made things worse.

That experience doesn't just create frustration; it teaches them never to reach out again:

"When I got extremely sick and had to go to *urgent care*, my employer *did not care*. No empathy or anything, just frustrated that I couldn't come in."

0%

of dismissed caregivers are very confident about staying



The early months are when this pattern gets set. First-year caregivers who feel supported are mostly confident about their future at the agency. Those who handle things alone are mostly unsure or unlikely to stay. The agency is building its reputation with each caregiver in real time, and the verdict is usually reached well before anyone in the office knows there's a problem.

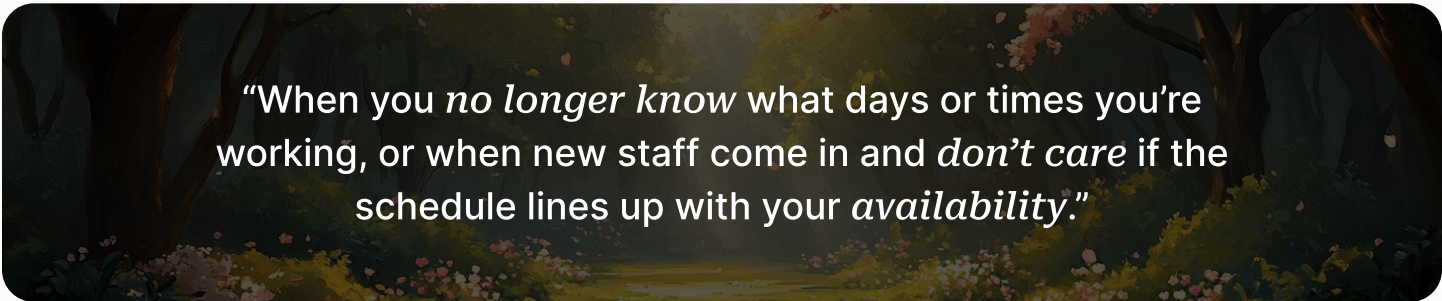
The first 90 days aren't a probationary period for the caregiver so much as a probationary period for the agency. Structured check-ins, early mentorship, and systems that make it easy for caregivers to reach out and fast for the office to respond aren't overhead costs but rather the highest-return retention investment available.

Scheduling, Income, and the Daily Experience That *Drives Them Out*

Scheduling is the daily rhythm of every home care agency, and caregivers understand that changes happen. What they don't accept is being treated poorly in the process. How agencies communicate schedule changes turned out to be the single strongest predictor of retention in the entire dataset, stronger than pay, hours, tenure, or leadership perception.

Caregivers who receive advance notice and options when schedules change are 62% very confident about staying. When they feel pressured to accept changes, that collapses to 7%. Among those who felt blamed or penalized for not being available, every single one is unsure or unlikely to stay.

The frustration caregivers describe isn't about changes happening so much as feeling invisible when they do:

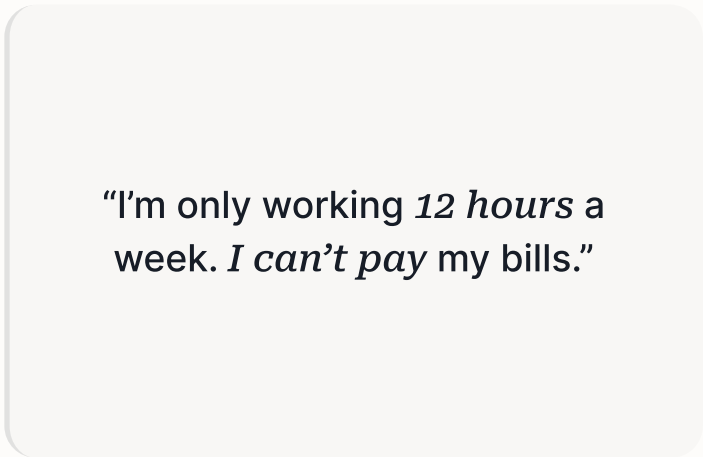


*"When you **no longer know** what days or times you're working, or when new staff come in and **don't care** if the schedule lines up with your **availability**."*

Across all caregivers who experienced negative scheduling communication, 59% think about leaving monthly or more and 70% are unsure about their future at the agency.

There's also a quieter signal embedded in this part of the data. Caregivers who start repeatedly declining shifts out of frustration are overwhelmingly unsure or unlikely to stay, and that behavior isn't a scheduling gap but a relationship gap. The signal is already sitting in most scheduling systems; the question is whether anyone is reading it.

Scheduling problems compound when they create financial instability, and for many caregivers, they do. More than 40% work fewer than 25 hours per week, and among those who constantly worry about income, 45% think about leaving monthly or more, nearly three times the rate of those with no financial pressure.



*"I'm only working **12 hours** a week. **I can't pay my bills**."*

For many, the math simply doesn't work:

When hours are this unstable, caregivers don't quit — they go find a second agency. Over half (51%) have worked for multiple agencies to stabilize their income, and most described this not as disloyalty but as survival:

“

“I like *consistency*, I love my clients. If the company can provide me with somewhat *stable hours*, with or *without* having a 2nd agency to guarantee my hours for the week.”

>50%

of caregivers have worked for multiple agencies to stabilize their income



But the income problem isn't just about hours. Even caregivers who have steady schedules described compensation that doesn't reflect what the work actually demands:

“I am not making a *living wage* even if *my hours* are 32 or more.”

Pay came up more than any other single topic when caregivers were asked the one thing that would make them more likely to stay — 36% named it, nearly double the next closest answer. And nearly half (49%) describe the stress of income instability as extremely or significantly affecting them.

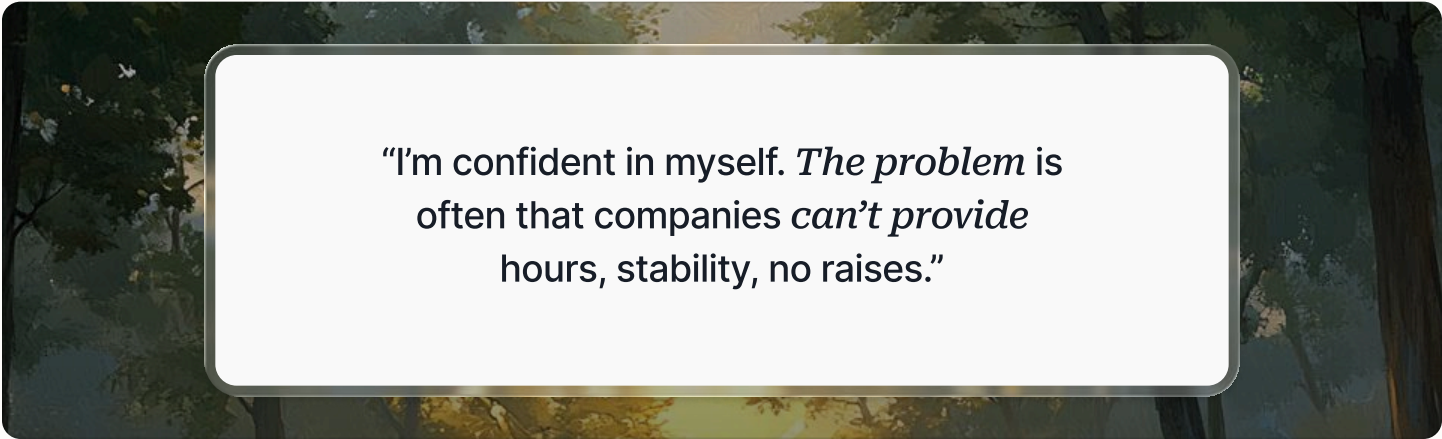
”

“The current pay is only almost *minimum wage* for the work that the *caregivers* do for *their clients*.”

The frustration isn't abstract. It's a caregiver doing physically and emotionally demanding work, showing up reliably, and still not being able to make ends meet. For some, the issue isn't just the rate — it's the feeling of being frozen in place:

“My pay has been *stagnant* for over 2 years.”

Among caregivers who constantly worry about income — a quarter of this dataset — 58% are at-risk for leaving, compared to 13% of those with no financial pressure. That's the widest confidence gap tied to any single experience in the data outside of scheduling communication.

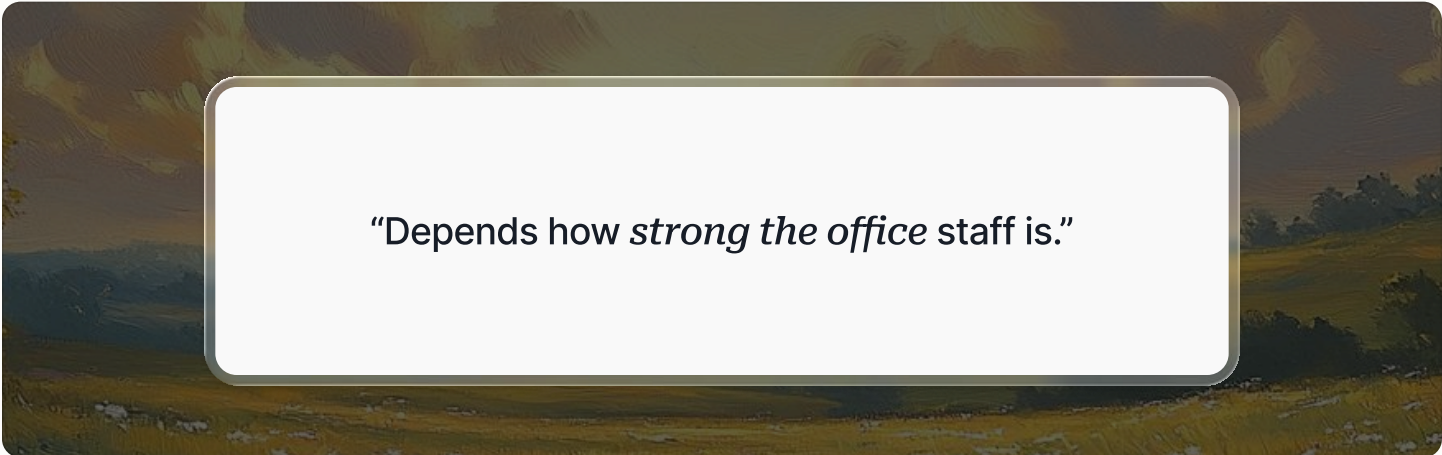


"I'm confident in myself. *The problem* is often that companies *can't provide* hours, stability, no raises."

Agencies that can't move on wages immediately can still close the gap by stabilizing hours, building schedules at least a week out, and being honest when hours fluctuate. But the caregivers in this dataset are clear: the ones that address both pay and stability together will see the strongest retention gains.

Leadership, Follow-Through, and the Trust That Holds Everything Together

When asked why they were confident about staying, one caregiver gave an answer that cuts to the core of what every data point in this section confirms:



"Depends how *strong the office* staff is."

More than half of caregivers describe their leadership as organized and proactive, and those caregivers are overwhelmingly confident about staying. But about a quarter describe leadership as inconsistent, disconnected, or overwhelmed. Among caregivers who see leadership as disconnected from caregiver realities, only 6% are very confident about staying and 71% are unsure or unlikely.

The mechanism is specific: it's follow-through. When leadership follows through on commitments consistently, 66% of caregivers are very confident about staying. When it happens about half the time, confidence drops to 22% and flight risk nearly triples. The threshold between trust and skepticism is lower than most leaders expect, and the drop-off is sudden.

Caregivers notice the gap between what's promised and what's delivered, even when they don't say anything about it. Especially when they don't say anything about it:

***"Making promises they don't keep.
Setting rules and not following through."***

And when trust erodes, it doesn't just affect how caregivers feel about leadership. It reshapes how they interact with the entire agency. They gradually stop raising concerns or asking for help, and start handling problems on their own. One caregiver described feeling like the people who were supposed to support her were part of the problem:

***"HR is hostile and
unapproachable."***

Others described an office culture where dishonesty had become the norm:

***"Backstabbing originating from the office, being lied to,
undue blame when things go wrong."***

And some described the moment they realized leadership had simply stopped trying, which was usually the moment they stopped trying too:

“When they *quit caring*.”

This connects directly to psychological safety, which is really about the quality of the relationship between caregiver and agency. Do I feel like I can be honest with these people? Will anything change if I raise a concern?

Caregivers who always feel safe raising concerns have a 12% at-risk rate. Those who only sometimes feel safe jump to 38%, more than triple the rate. The caregivers who don't feel safe speaking up are the hardest to see coming, because they never complain or make demands. They quietly stop investing in the relationship, start handling problems on their own, and eventually walk away.

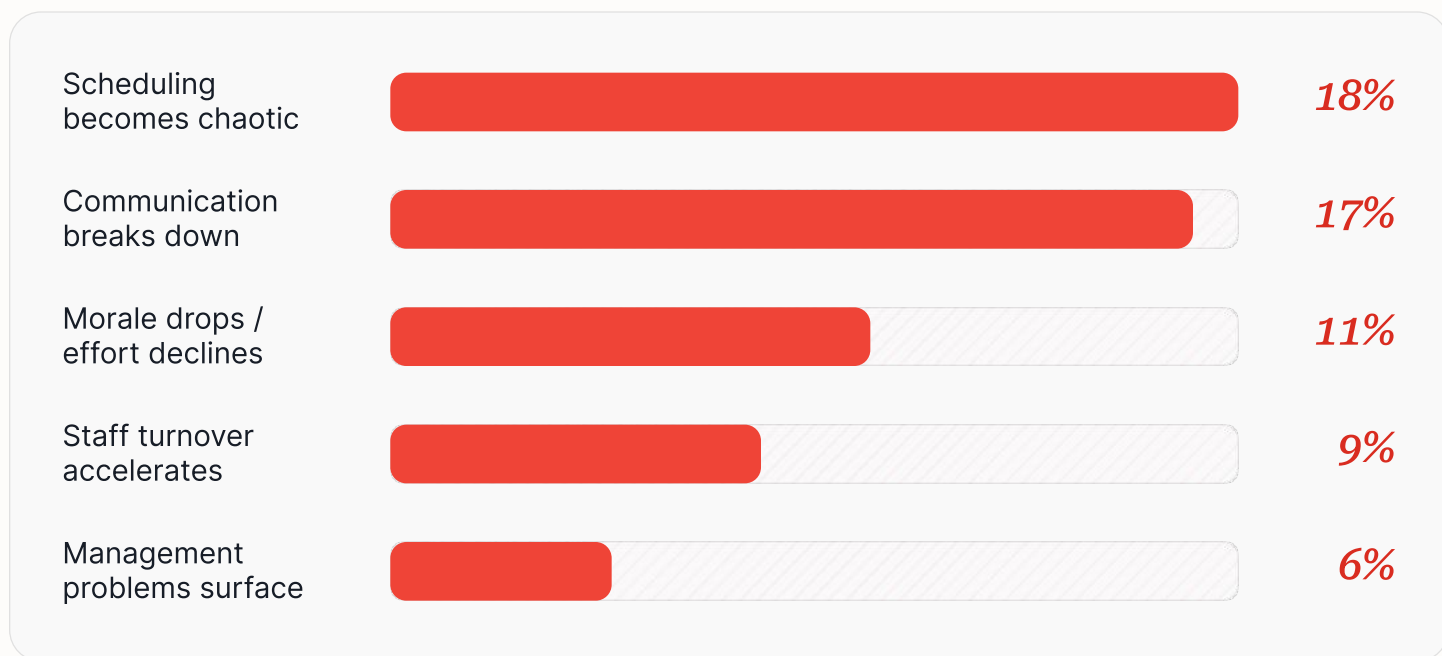


When asked what would make them more likely to stay, one caregiver didn't ask for more money or better hours but for something more fundamental:

“Knowing that if I have an issue I can reach out without being scared.”

Caregivers are also remarkably clear-eyed about recognizing when an agency is starting to falter, often before leadership sees it themselves:

Early warning signs *caregivers notice*



Follow-through is a measurable behavior with a direct line to retention. The agencies with the strongest retention will be the ones where caregivers feel genuinely safe raising concerns, and where those concerns are met with visible action. Building that environment costs nothing but consistency, and the cost of not building it is invisible until it's too late.

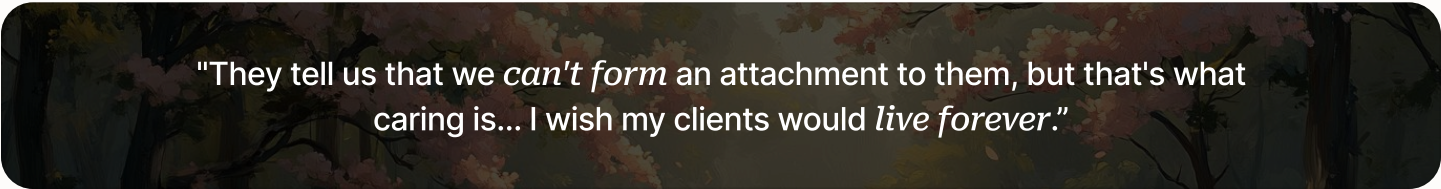
What Keeps Them Isn't What *They Ask You to Fix*

This is one of the most important findings in the report, and it requires holding two truths at the same time.

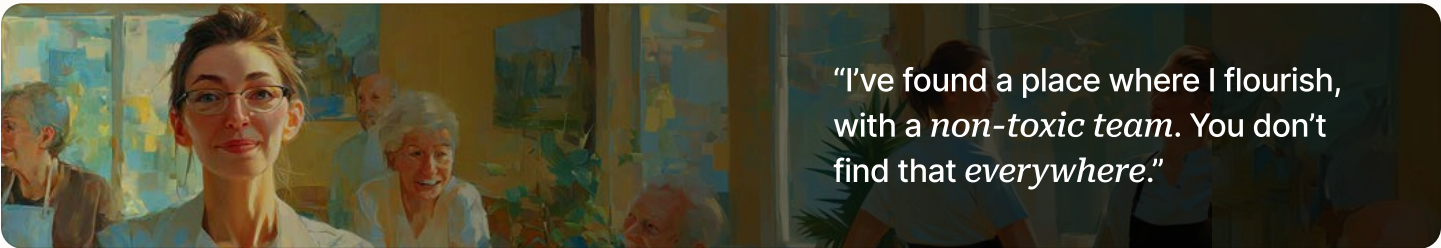
When caregivers talk about why they stay, the answers are personal. Supportive culture and team lead the list, followed by client relationships, then schedule flexibility and pay. They stay because they feel valued by the people they work with and connected to the people they care for. That loyalty often runs deep, but it's directed at clients, not necessarily the agency:

"I am a person who is *loyal with my clients*.
Not necessarily the agency, but my clients, and
I can't *bring myself* to just up and leave them."

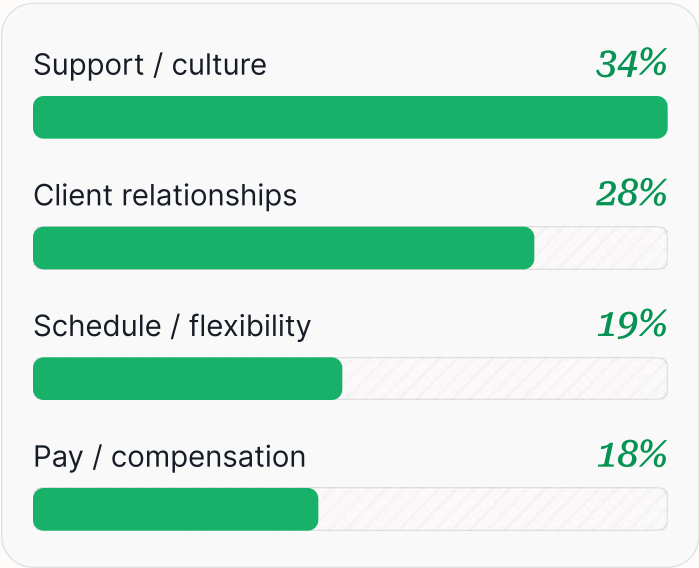
The bond with clients runs deeper than most agencies acknowledge, and caregivers feel caught between what the job asks of them emotionally and what they're told is professional:



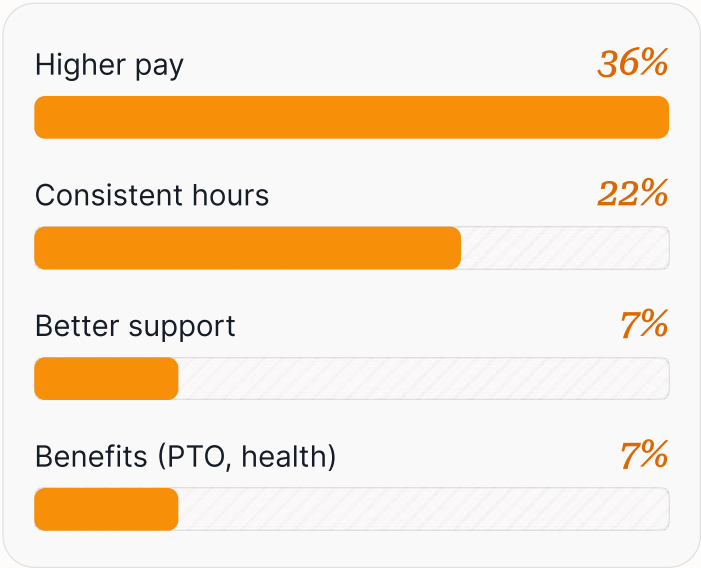
And when the team and culture are right, caregivers recognize it and hold onto it:



Why they stay



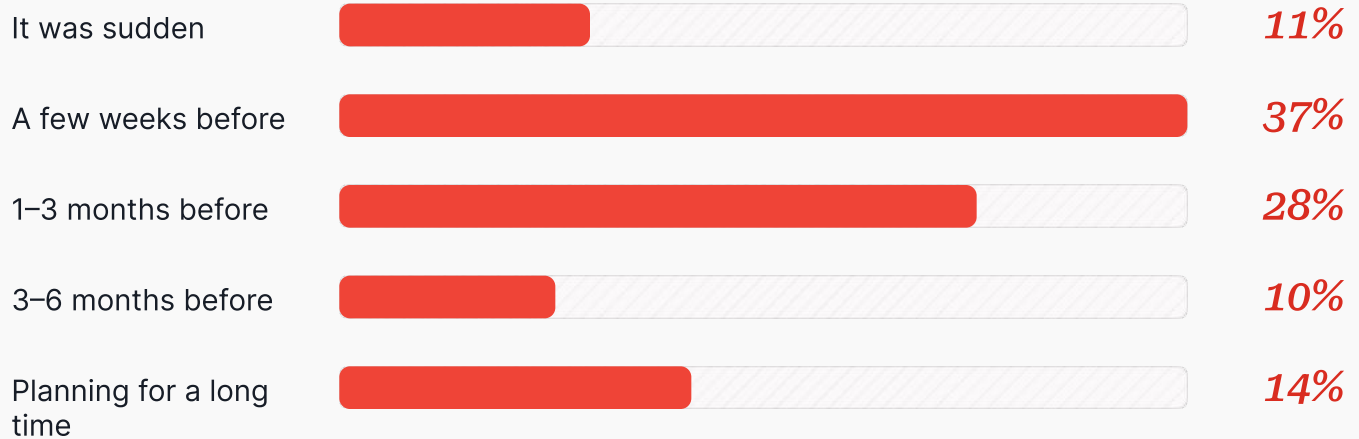
Why they'd change



Pay is the number one thing caregivers want fixed, but it's not the number one reason they stay. Agencies that raise wages but neglect culture will slow turnover temporarily. Agencies that raise wages and invest in the relationships and systems that create real loyalty will see retention compound over time.

But understanding what keeps caregivers also means understanding how quickly the window closes when those things break down. The decision timeline tells the story:

When caregivers mentally *decided to leave*



Nearly 90% of caregivers who left had made their decision weeks or months before anyone in the office knew. Only 11% described the departure as sudden. The rest were already gone emotionally — still showing up, still caring for clients, but no longer invested in the agency. That gap between the internal decision and the external notice is where retention is won or lost, and for most agencies, no one is watching for it.

But when agencies do get it right, caregivers notice just as clearly:

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“My company is *very compassionate* and they tell us all the time how much they *appreciate us*. I can’t imagine *working for a better company ever*.”

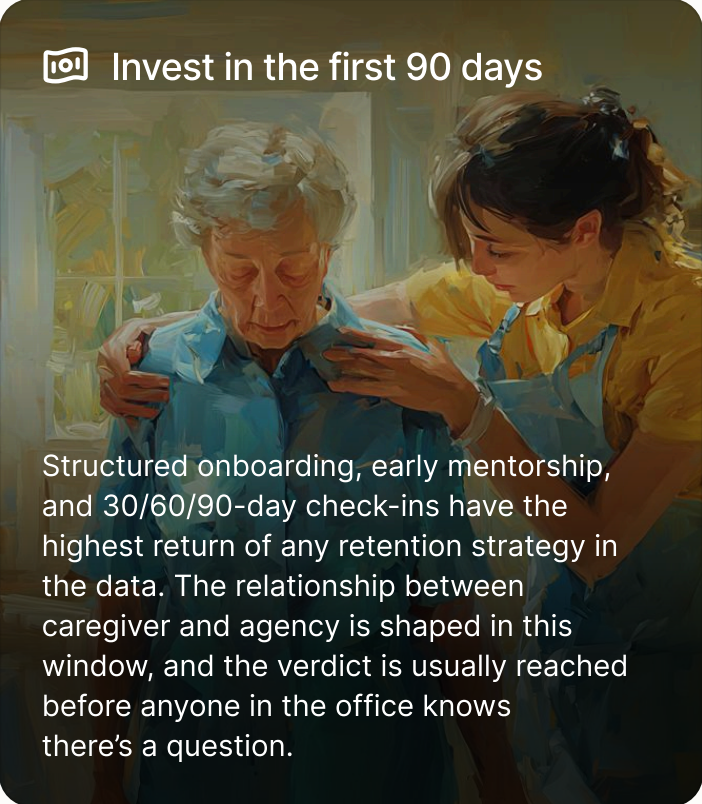
And for many, the work itself remains deeply fulfilling, which is exactly why losing good caregivers to fixable problems is so costly:

“I love what I do. I do my job to *help others* to better their *quality of life*. It really makes *my heart happy*.”

What To Do *With This Report*

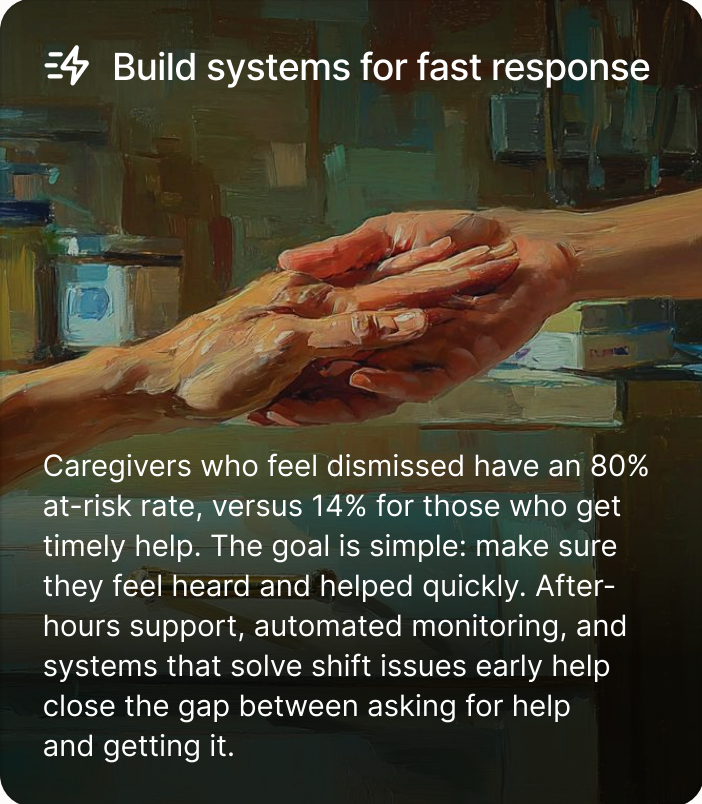
6 things you can do right now to make your caregivers more engaged

Invest in the first 90 days



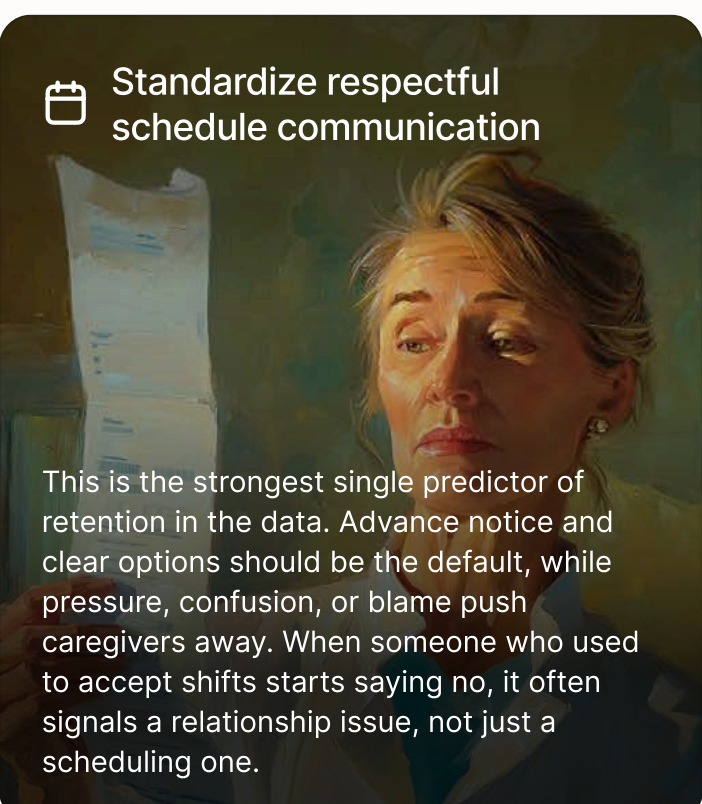
Structured onboarding, early mentorship, and 30/60/90-day check-ins have the highest return of any retention strategy in the data. The relationship between caregiver and agency is shaped in this window, and the verdict is usually reached before anyone in the office knows there's a question.

Build systems for fast response



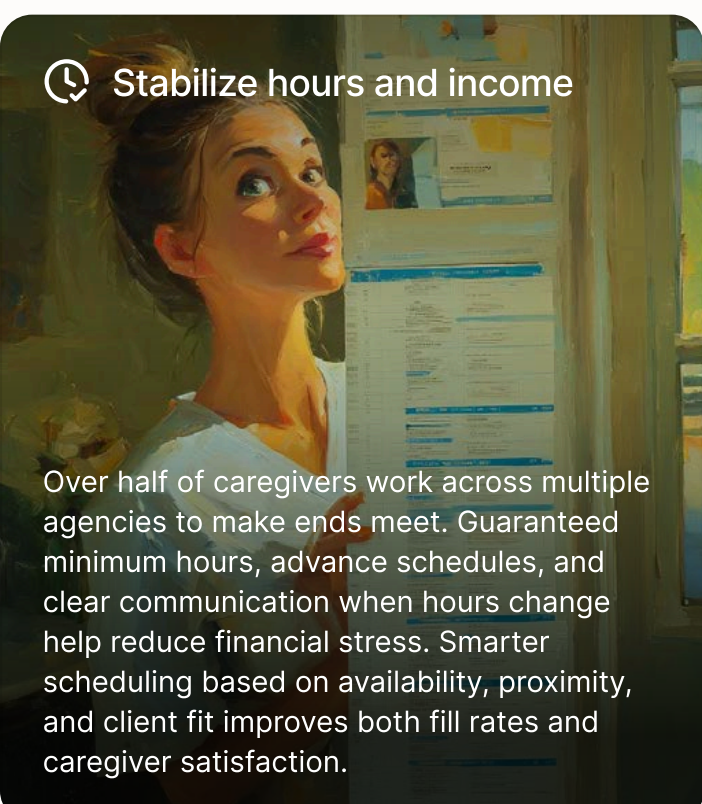
Caregivers who feel dismissed have an 80% at-risk rate, versus 14% for those who get timely help. The goal is simple: make sure they feel heard and helped quickly. After-hours support, automated monitoring, and systems that solve shift issues early help close the gap between asking for help and getting it.

Standardize respectful schedule communication

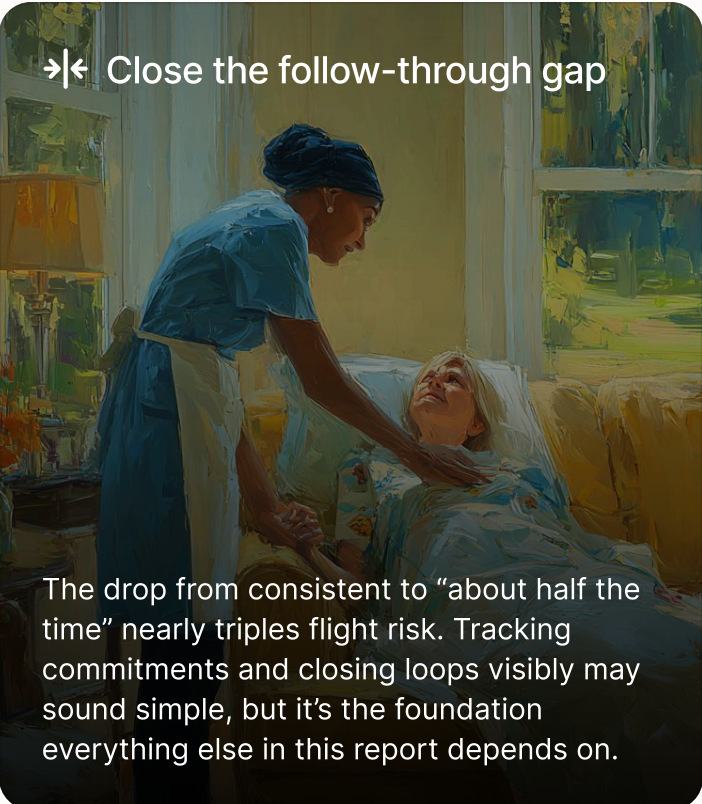


This is the strongest single predictor of retention in the data. Advance notice and clear options should be the default, while pressure, confusion, or blame push caregivers away. When someone who used to accept shifts starts saying no, it often signals a relationship issue, not just a scheduling one.

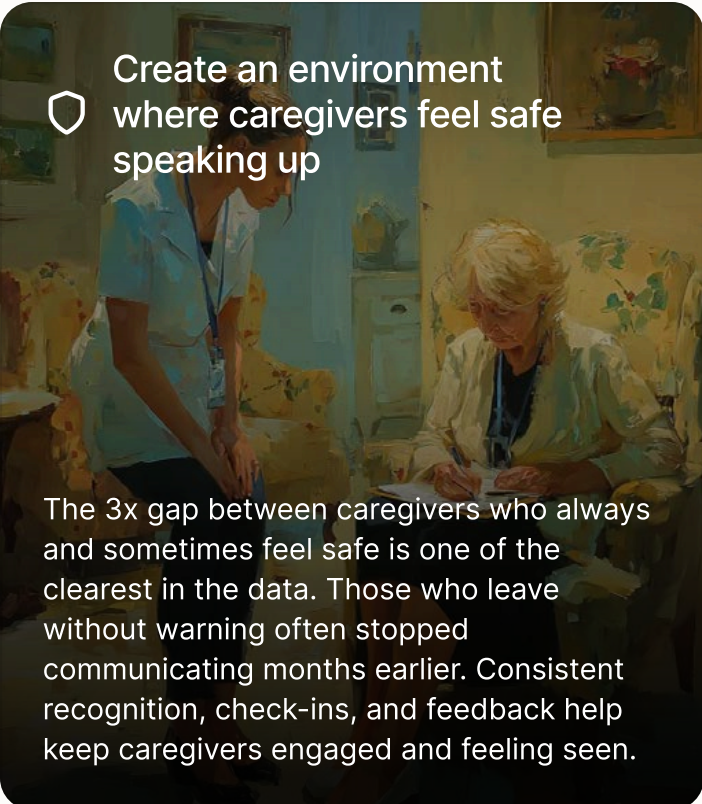
Stabilize hours and income



Over half of caregivers work across multiple agencies to make ends meet. Guaranteed minimum hours, advance schedules, and clear communication when hours change help reduce financial stress. Smarter scheduling based on availability, proximity, and client fit improves both fill rates and caregiver satisfaction.

 Close the follow-through gap

The drop from consistent to “about half the time” nearly triples flight risk. Tracking commitments and closing loops visibly may sound simple, but it’s the foundation everything else in this report depends on.

 Create an environment where caregivers feel safe speaking up

The 3x gap between caregivers who always and sometimes feel safe is one of the clearest in the data. Those who leave without warning often stopped communicating months earlier. Consistent recognition, check-ins, and feedback help keep caregivers engaged and feeling seen.

 Invest in pay and culture together

Pay is the top requested fix, but client relationships, team culture, and feeling valued are what actually keep people. The agencies that do both will see retention compound over time in ways that neither strategy achieves alone.

About This Report

This report summarizes findings from ~150 responses to a 2026 caregiver experience survey fielded to active and recently departed home care workers across multiple U.S. agencies.

Results are presented as benchmarks of experience, perception, and retention risk based on respondent selections to structured questions, supplemented by open-ended responses.

Quantitative data was analyzed by frequency distribution and cross-tabulation. Correlations were computed using segmented analysis across key variables including support experience, income stress, leadership perception, psychological safety, follow-through, and schedule communication.

The compound risk score is an unweighted additive index of five binary factors. Not every respondent answered every question; percentages are calculated among respondents who answered each question.

Quotes are drawn from open-ended survey responses and have been lightly edited for clarity. Nearly half of respondents (48%) have five or more years of total home care experience, and 16% have fifteen years or longer.